



APPEALFORGE PAC

Denial-to-appeal evidence assembly for post-acute Medicare Advantage teams

A polished, source-cited workflow built for SNF/IRF/LTCH operators facing Medicare Advantage admission and continued-stay denials.

95%

OIG SNF appealed denials overturned in sample

10m

Target appeal packet draft time

0

Uncited clinical claims allowed

Packet Preview

Re: Expedited reconsideration

The denial states the requested SNF admission does not meet skilled level of care.

The attached evidence supports daily skilled nursing and therapy needs.

Draft · citations required

The critical unsolved problem

Winnable post-acute denials are not operationally appealable fast enough.

The appeal is often not blocked by clinical facts. It is blocked by document chaos, deadline pressure, and staff bandwidth.

Denial letter gives a reason, but evidence is buried across PDFs and notes.

UR/admissions teams copy, paste, fax, and chase signatures under deadline.

Physicians will not sign weak packets without clear source support.

Outcome data is rarely captured, so ROI and payer patterns stay invisible.

The exact wedge

Generate a source-cited Medicare Advantage reconsideration packet for denied SNF admission or continued stay in under 10 minutes.

Appeals first, not broad prior auth automation.

The signal is strong

The market data points to a last-mile appeal execution gap.

52.8M

MA prior authorization
determinations in 2024

4.1M

Denied in full or in part

11.5%

Denied requests
appealed

>80%

Appealed denials
overturned in 2019-2024

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Post-acute SNF wedge

HHS OIG found that in a June 2024 sample, 19 MAOs denied 12% of SNF admission requests. Only 18% were appealed. When appealed, 95% were overturned.

Product implication

Do not build generic PA software first. Build the narrow evidence-to-packet workflow where the denial reason, deadline, and outcome are measurable.

The product workflow

One guided path from denial letter to reviewed packet.

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1 Create case

Facility, plan, service, denial date.

2 Upload packet

Denial + clinical PDFs.

3 Parse denial

Reason, deadline, appeal route.

4 Map evidence

Criteria -> source quotes.

5 Resolve gaps

Missing evidence checklist.

6 Export packet

Human-reviewed PDF.

Design should create trust

Make users want to login - premium, calm, precise.

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Midnight clinical atelier

Dark command-center shell.
Warm paper packet preview.
Teal for action.
Amber for deadline.
Coral only for true risk.
Source quotes as first-class UI.

Evidence card

Evidence found

Needs review

Unsupported claim blocked

Not a default dashboard. A clinical evidence workbench.

The evidence map is the hero screen

It turns denial rationale into source-backed review work.

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Daily skilled nursing need

2 citations · reviewed

Skilled therapy need

1 citation · needs note

Lower level unsafe

blocked · no source

Physician statement

missing · owner:
Medical Director

Source quote drawer

"Patient requires moderate assistance for transfers and is unable to ambulate safely without skilled cueing."

Therapy Eval · page 4
Supports: skilled therapy need, fall risk, lower-level rebuttal.

Safety is the product moat

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AppealForge should be safer than manual copy/paste.

Citation-only claims

Every clinical assertion traces to document id, page, and quote.

Human approval

Drafts require staff/clinical/physician review before approved export.

Missing evidence gaps

Absence becomes a task, not a hallucination.

Audit-ready

Sensitive actions are logged without PHI in metadata.

No proprietary criteria misuse

Use public/general or customer-licensed criteria only.

Architecture

Buildable by Codex as a local demo, production-shaped for healthcare.

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Next.js 15 + TypeScript

Polished app shell, case screens, evidence map, packet editor.

Prisma + PostgreSQL

Organization/facility scoped healthcare workflow data.

Object storage adapter

PDFs and exported packet versions.

Worker jobs

Extraction, parser, evidence, packet generation, export.

AI pipeline

Structured outputs with citation validators and claim blocker.

Audit/RBAC

PHI-safe security posture from day one.

MVP scope

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No payer portal automation. No EHR integration. Prove the packet workflow first.

Build now

- Login/demo mode
- Case dashboard
- Upload + extraction mock
- Denial parser review
- Evidence map
- Missing evidence checklist

Do not build yet

- Autonomous submission
- Payer portal scraping
- Full EHR integration
- Live PHI demo
- Consumer app
- Claims adjudication

GTM wedge

Start with regional operators and historical packet proof.

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ICP

Regional SNF/post-acute
operators
3-50 facilities
High MA census
Recurring
admission/continued-stay
denials

Pilot

90 days
\$1.5k-\$3k per facility/mo
De-ID historical cases first
Production only after BAAs

Proof

Packet time reduced
More denials appealed
Physician trust
Outcome/ROI captured

14-day validation

Stress test before overbuilding.

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Days 1-2

Target list

Days 3-7

**20
discovery
calls**

Days 8-10

**3-5 de-ID
packet
tests**

Days 11-12

**Demo
back**

Days 13-14

**Pilot
decision**

Pass only if at least five operators confirm recurring MA denials, painful appeal assembly, some appeal wins, de-identified data access, and pilot budget ownership.

Codex execution plan

Tokenmax the build so it does not stop at a scaffold.

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Combined sprint

Use SPRINT_00_COMBINED_ULTRA.md.
It forces `.codex/progress.md`, end-to-end screens, seed data, tests, export, audit, and polish.

Anti-premature-stop rule:
not done after scaffolding;
not done after static pages;
not done after one green build.

Fallback prompts

- SPRINT_01_FOUNDATION
- SPRINT_02_UI_DESIGN
- SPRINT_03_DOCUMENT_AI_WORKFLOW
- SPRINT_04_EVIDENCE_PACKET
- CONTINUE_PROMPT
- REVIEW_POLISH_PROMPT